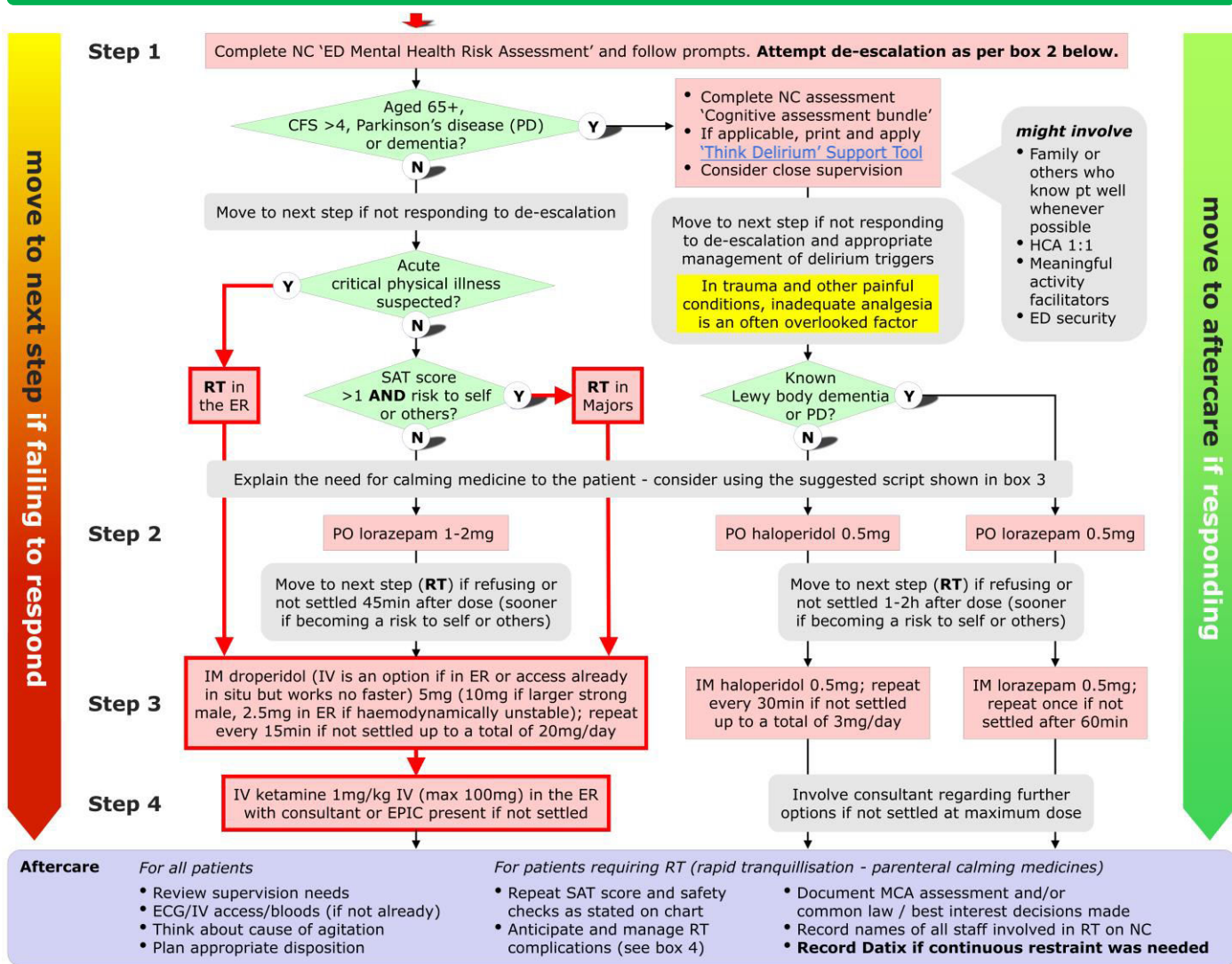


① Acute behavioural disturbance (ABD) management including rapid tranquillisation (RT)

To prescribe in NC Meds, go to Emergency Medicine (ED) > Common scenarios (ED) > Acute behavioural disturbance



② De-escalation *the actions below can help reduce the need for physical restraint and injectable RT medicines*

[REF]
link

Maintain self-control

Exposure to aggression can affect your emotion; stay calm and in control.

Respect patient's personal space

Give extra space to reduce the sense of threat.
Patients like having room to move around.

Show empathy

Show understanding through words and body language.

Reassure

Fear may drive aggression. Let patients know they're safe, respected, and won't be harmed.
Patients often feel safer with familiar staff.

Avoid shaming

Shame can escalate aggression. Preserve dignity.
Patients want to be heard, spoken to kindly, and treated with respect.

Avoid provocation

Steer clear of known triggers and avoid escalation.

Don't act over the patient's head

Keep communicating, even if it's difficult.

Never appear to use brute force or aim to punish

Frame treatment as supportive, not as punishment.

Set limits in a nonconfrontational way

Stay calm and offer choices; avoid threats or ultimatums.

Use passive intervention and watchful waiting

Keep patient's cognitive load and demands on concentration minimal while observing them closely.

Manage the environment

Change setting or suggest calming activities to lower stress.

Use distraction

Shift attention to reduce distress.

Patients enjoy access to art, music, and other distractions.

Identify patient's needs

Aggression often signals unmet needs – including pain relief. If applicable, use 'Think Delirium' support tool to find triggers.
*Pts value food and drink (and often also a chance to smoke).
Manage withdrawal from nicotine / alcohol / opiates etc.*

Negotiate

Seek shared goals. Control issues often drive aggression.
Patients want oral medicine options they can choose from.

Use gentle humour

When used sensitively, this can ease tension.

Reframe events for patient

Emotions arise from making judgements about the motivation of others. Help the patient to consider other explanations for what is, or has been, happening.

Assess risk dynamically

Regularly reassess the level of risk posed by the patient and how they are responding to staff.